

MP-IMG-001 Lumbar Spine MRI

1 Purpose and scope

This policy describes when Meridian Health Plan considers magnetic resonance imaging (MRI) of the lumbar spine medically necessary. It applies to CPT codes 72148 (without contrast), 72149 (with contrast) and 72158 (without and with contrast). Meridian Health Plan is a fictional payer created for a portfolio project, and this document is not clinical guidance.

2 Definitions

Conservative therapy means at least 6 consecutive weeks of treatment that combines two or more of the following: supervised physical therapy or a documented home exercise program, prescription or over-the-counter anti-inflammatory or analgesic medication, activity modification, and spinal manipulation or chiropractic care. Radiculopathy means pain radiating into a limb in a nerve-root distribution, with or without objective neurological findings. Objective neurological findings include a positive straight leg raise, dermatomal sensory loss, motor weakness or a reflex change.

3.1 Red-flag conditions

Lumbar MRI is approved without a trial of conservative therapy when the clinical note documents any of the following:

- known or suspected cancer, including new back pain in a patient with a history of malignancy
- suspected spinal infection, such as fever with back pain or recent intravenous drug use
- signs of cauda equina syndrome, including urinary retention, bowel incontinence or saddle anesthesia
- progressive or severe motor weakness
- suspected fracture after significant trauma

Requests with red-flag conditions qualify for expedited review.

3.2 Radiculopathy with neurological findings

Lumbar MRI is approved for radiculopathy when the patient has completed at least 6 weeks of conservative therapy without adequate improvement and the note documents at least one objective neurological finding. The request should state how long each treatment was tried and the patient's response.

3.3 Persistent low back pain without radiculopathy

Lumbar MRI is approved for low back pain without radicular symptoms only when pain persists after at least 6 weeks of conservative therapy and the treating clinician documents that the result

would change management, for example a planned epidural injection or a surgical referral.

3.4 Prior lumbar surgery

Lumbar MRI is approved for a patient who has had lumbar surgery when new or worsening symptoms appear, such as recurrent radicular pain, new weakness, or suspected infection or hematoma. Routine surveillance imaging without new symptoms is not approved.

4 Exclusions

Lumbar MRI is not considered medically necessary in these situations:

- acute nonspecific low back pain of less than 6 weeks duration without red-flag conditions
- a repeat MRI within 12 months when there is no change in clinical status and no new neurological finding
- imaging requested only for patient preference or medico-legal reasons
- pain that has improved with conservative therapy

5 Required documentation

The request must include the duration of symptoms; each conservative treatment tried, with start date, duration and response; neurological examination findings; whether red-flag conditions are present or denied; results of any prior spine imaging; and the CPT and ICD-10 codes. Requests that omit these items are pended for additional information.